IN THE SUPREME COURT OF IOWA
No. 25-1433

MICHAEL WOOFF,
Appellant,

vs.

NINE PARK LANE, INC. d/b/a MENEFEE DRYWALL and WESTERN NATIONAL INSURANCE COMPANY,
Appellees.

APPEAL FROM THE IOWA DISTRICT COURT FOR LINN COUNTY
THE HONORABLE CHAD KEPROS
NO. CVCV106242

APPELLEES' BRIEF

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STATEMENT OF THE ISSUES PRESENTED FOR REVIEW

I. Whether the Agency committed an error of law in concluding that Claimant is not entitled to industrial disability benefits.

II. Whether substantial evidence supports the Agency's decision that Claimant is not entitled to industrial disability benefits due to his sequela mental injury.

III. Whether the Agency erred as a matter of law in determining the extent of permanent impairment.

ROUTING STATEMENT

This case involves the application of existing legal principals and should be transferred to the Court of Appeals pursuant to Iowa Rules of Appellate Procedure 6.1101.

NATURE OF THE CASE

Appellant Michael Wooff (hereinafter "Claimant) alleges he sustained a cumulative injury to his right shoulder and neck and a sequela mental injury due to his work duties in drywalling with a manifestation date of August 9, 2019. Appellees (hereinafter "Defendants") accepted compensability of the right shoulder injury but denied compensability of the neck and mental injuries. Claimant filed a Petition with the Iowa Workers' Compensation Commissioner on

August 11, 2022, claiming entitlement to workers' compensation benefits. This matter came for an arbitration hearing before Deputy Andrew Phillips on October 4, 2023.

In the Arbitration Decision filed on March 21, 2024, Deputy Phillips held that Claimant proved that he sustained a cumulative neck injury that arose out of and in the course of his employment with Defendants. (Arb. Dec. p. 49). He further held that Claimant proved his mental condition was aggravated or lit up by the right shoulder and neck injuries. (Arb. Dec. p. 52). Claimant was awarded temporary total disability benefits from October 18, 2021 through April 21, 2022 and March 15, 2023 through June 24, 2023 for his neck injury. (Arb. Dec. p. 53). Deputy Phillips found that Claimant was not permanently and totally disabled or odd lot. (Arb. Dec. pp. 56-57). He awarded Claimant 55% industrial disability benefit commencing on June 25, 2023. (Arb. Dec. p. 60). In regard to medical expenses, Deputy Phillips ordered Defendants to reimburse Claimant $244,133.02 for medical care; he did not order reimbursement for several emergency room visits and noted there was no indication that the claimant attempted to contact Defendants for authorized medical care prior to go to the emergency room. (Arb. Dec. pp. 62-63). Claimant was awarded 35% penalty benefits on the alleged late payments of weekly benefits. (Arb. Dec. 65). Claimant was awarded

reimbursement of his IME with Dr. Segal and case costs of $4,118.64. (Arb. Dec. p. 67).

Defendants filed an appeal of the Arbitration Decision to the Iowa Workers' Compensation Commissioner on April 8, 2024. Claimant filed a Motion for Rehearing on April 9, 2024, requesting clarification regarding which medical expenses were to be reimbursed. The deputy issued a Rehearing Decision and Nunc Pro Tunc Order on April 11, 2024. Claimant filed a notice of cross appeal on April 28, 2024.

In the Appeal Decision filed on October 18, 2024, the Commissioner affirmed that Claimant proved he sustained an injury to his right shoulder, neck, and a sequela mental injury. (App. Dec. p. 2). The Commissioner reversed the deputy's finding regarding industrial disability. (App. Dec. p. 4). The Commissioner determined that the claimant was not entitled to industrial disability because Defendants offered work at the same or greater wages, which the claimant declined. (*Id.*). The Commissioner adopted Dr. Chen's impairment rating for the right shoulder, which was 9% upper extremity impairment or 5% whole body impairment. (App. Dec. p. 5). He adopted Dr. Segal's impairment rating of 35% whole body impairment for the neck. (*Id.*). The Commissioner determined Claimant did not sustain any functional impairment under the AMA Guides due to his mental health injury. (*Id.*). The Commissioner then added a 9% whole body

impairment rating with a 35% whole body impairment rating using the Combined Values Chart of the AMA Guides and stated Claimant had sustained functional disability of 41% whole body impairment. (App. Dec. p. 6). Therefore, Claimant was awarded 205 weeks of permanent partial disability benefits. (*Id.*). The Commissioner ordered Defendants to reimburse Claimant for the additional emergency room visits of June 27, 2022, September 14, 2022, and October 6, 2022. (App. Dec. p. 8). Claimant was awarded $2,500 in penalty benefits. (App. Dec. p. 10).

Claimant filed a Motion for Rehearing with the Commissioner on November 6, 2024. On November 12, 2024, the Commissioner denied the Motion for Rehearing. Defendants filed a Request for a Nunc Pro Tunc Order on November 13, 2024 regarding the Commissioner's erroneous calculation of permanent functional disability. The Commissioner denied the Request on November 26, 2024, stating that it should have been filed as a request for rehearing rather than a request for a nunc pro tunc order.

Claimant filed a Petition for Judicial Review with the Polk County District Court on November 14, 2024. Defendants filed an Answer and Cross Petition on December 12, 2024. Defendants filed an Amended Cross Petition on February 7, 2025.

In the Ruling on the Petition for Judicial Review filed on July 14, 2025, Judge Kepros upheld the Commissioner's determination that Claimant is not entitled to industrial disability benefits as Defendants provided a good faith offer of employment. (Ruling p. 12). He determined that even if Claimant was entitled to industrial disability benefits, there was substantial evidence in the record to support the Commissioner's decision that Claimant's mental health condition would not increase the industrial disability award. (*Id.*). Judge Kepros concluded that the Commissioner did not commit an error of law or improperly use Agency expertise in determining the award of functional disability benefits. (Ruling p. 13). Judge Kepros upheld the Agency decision that Claimant sustained a compensable neck and mental health sequela injury. (Ruling pp. 13-14). In regard to the calculation of permanent impairment for the neck and shoulder, the Court ordered Defendants to submit a proposed order regarding the issue. (Ruling p. 14).

Judge Kepros issued an Order regarding the calculation of whole-body impairment on August 5, 2025. He stated that the Agency's calculation of whole-body impairment for the right shoulder and neck is 38% whole body impairment when using the Combined Values Chart of the AMA Guides. (Order p. 2).

Claimant filed a Notice of Appeal with the Iowa Supreme Court on September 3, 2025.

STATEMENT OF THE FACTS

I. Background Information

At the time of the hearing, Claimant was 56 years old. (Hr. Tr. p. 19). He graduated from high school in 1986. (Ex. 4, p. 155). Claimant was in the United States Marine Corps from 1987 to 1991 and received training as a motor transport operator. (*Id.*). Claimant worked for Interstate Battery from 1991 to 1992. (Ex. 4, p. 156). He then worked as a screen printer from 1986 to 1987 and again in 1992. (Ex. 4, pp. 156-57). Claimant started working for Menefee Drywall in August 1993. (Ex. 4, p. 156). Claimant worked as a finisher of drywall; he would tape, mud, and sand the drywall. (Hr. Tr. p. 23). Claimant stated he was familiar with the entire drywall process and knew how to identify whether drywall was installed correctly. (Hr. Tr. pp. 58-59).

Claimant stated there was no specific incident that caused his right shoulder pain. (Hr. Tr. p. 26). "It was just getting to a point where I couldn't handle the pain anymore." (*Id.*). Claimant reported his right shoulder pain to his supervisor. (*Id.*).

After Claimant was released from treatment with permanent work restrictions, Menefee Drywall offered Claimant a job that was consistent with his permanent work restrictions on August 25, 2021. (Ex. E, p. 69). The position was a superintendent position that paid $60,000 annually plus benefits. (*Id.*). Claimant didn't think he could perform that job because it required computer work. (Hr. Tr.

p. 42). Claimant stated he was also offered the position of commercial supervisor, but he declined the job because he didn't feel comfortable performing the job. (Hr. Tr. pp. 42-43). Claimant last worked for Defendants on September 30, 2021. (Hr. Tr. p. 60).

Claimant worked for Meth-Wick Community from March 2022 through October 2022. (Ex. 4, p. 156). He did light maintenance work at a retirement facility. (Hr. Tr. p. 40). Claimant passed a preemployment physical prior to starting the job. (Hr. Tr. p. 64). Claimant testified that he was unable to perform the work as it caused his pain to increase. (Hr. Tr. p. 41). Claimant has not applied for any jobs since then. (Hr. Tr. p. 65).

Claimant stated he currently has right shoulder pain that he rates as 6/10. (Hr. Tr. p. 37). He stated he has dull pain in his neck. (Hr. Tr. p. 36). He does not take any pain medications. (Hr. Tr. p. 38). Claimant stated his restrictions related to his right shoulder are to lift what he is able and avoid repetitive reaching away from the body or above shoulder height. (Hr. Tr. p. 66). Claimant isn't aware of any restrictions for his neck. (Hr. Tr. p. 67).

II. Medical Treatment

On September 6, 2019, Claimant presented to the Iowa City VA emergency department stating that his major depressive symptoms were progressively getting

worse. (Ex. JE1, p. 4). Claimant stated he had been battling depression symptoms for years and nothing was making him feel better. (*Id.*). Claimant was currently taking fluoxetine 20 mg. (*Id.*). Claimant thought he was doing better in July, but now his symptoms were getting worse. (*Id.*). Claimant reported lack of interest, severe sadness, and tearfulness without any specific triggering factor. (*Id.*). Claimant stated he started drinking more because of his symptoms and he was drinking around a 12 pack per day, 5 days a week. (*Id.*). Claimant noted he was still able to work; he had no problem finishing work but started drinking after work. (*Id.*). Claimant reported worsening sleep quality and having vivid nightmares related to war and invasion. (Ex. JE1, pp. 4-5). Claimant stated this was the worst he had ever been, and he was not capable of dealing with his alcohol dependence as an outpatient. (Ex. JE1, p. 5). Claimant was admitted to the psychiatric unit. (Ex. JE1, p. 6).

Claimant reported on September 6, 2019, while being admitted to the psychiatric unit that he had been struggling with depressive symptoms for "many years" with poor response to antidepressant medications. (Ex. JE1, p. 12). He noted his alcohol use had increased over the last year from a 6 pack per day to 12+ pack per day. (*Id.*). Claimant further reported he felt quite tired during the day. (Ex. JE1, p. 14). He stated his sleep was poor with snoring and frequent nighttime awakening, sometimes from nightmares but not always. (*Id.*). Dr. Wassink

increased the fluoxetine to 40 mg per day and prescribed trazadone for sleep. (Ex. JE1, p. 16). Claimant was discharged on September 7, 2019. (Ex. JE1, p. 21).

Claimant underwent a right shoulder MRI on September 20, 2019, which indicated a circumferential tear of the labrum and tearing of the supraspinatus and infraspinatus tendons. (Ex. JE3, p. 86).

On October 4, 2019, Claimant presented to Dr. Matthew White at Physicians' Clinic of Iowa for evaluation of his right shoulder. (Ex. JE4, p. 116). Dr. White explained that Claimant had a significant rotator cuff tear and changes within the labrum that were causing his symptoms. (Ex. JE4, p. 117). Dr. White recommended surgery. (*Id.*). Claimant underwent a right shoulder arthroscopic rotator cuff repair, debridement of the labrum, subacromial decompression, and mini open biceps tenodesis with Dr. White on October 28, 2019. (Ex. JE5, p. 127).

When Claimant followed up with Dr. White on December 11, 2019, he reported significant pain along the superior aspect of the shoulder that was quite limiting. (Ex. JE4, p. 122). He felt that his pain was worse than prior to surgery. (*Id.*). The pain was from his neck down the arm towards the elbow. (*Id.*). Dr. White questioned if Claimant's pain was cervical related and recommended a cervical spine MRI. (Ex. JE4, p. 123).

On December 26, 2019, Claimant underwent a cervical spine MRI that indicated mild diffuse degenerative changes at C3-4 causing mild to moderate right neural foraminal narrowing. (Ex. JE5, p. 134).

On January 15, 2020, Claimant reported to Dr. White that he had ongoing pain that was worse than prior to surgery. (Ex. JE4, p. 124). Claimant stated the pain extended into his neck and down the arm to the elbow. (*Id.*). Dr. White reviewed the MRI imaging and noted scattered degenerative changes with significant right-sided foraminal narrowing at C3-4. (*Id.*). Dr. White stated Claimant's pain could be due to the foraminal narrowing; Claimant's exam was not concerning for recurrent cuff abnormality or changes of the biceps. (Ex. JE4, p. 125). Dr. White stated he was unable to opine as to causation of the neck symptoms other than that Claimant didn't have the symptoms prior to his right shoulder surgery and his symptoms could be positional, block related, or therapy related. (*Id.*).

Claimant presented to Dr. Matthew Bollier at the University of Iowa Hospitals & Clinics for evaluation of his symptoms on January 31, 2020. (Ex. JE7, p. 250). Dr. Bollier assessed Claimant with adhesive capsulitis and recommended surgery. (*Id.*). Claimant underwent right shoulder arthroscopy, capsular release, extensive debridement, subacromial decompression, and distal clavicle excision with Dr. Bollier on February 18, 2020. (Ex. JE7, p. 251).

On June 15, 2020, Claimant reported significant worsening right shoulder pain that was also related to his neck and headaches. (Ex. JE7, p. 275). Dr. Bollier did not believe Claimant needed any additional shoulder imaging and stated Claimant's overall shoulder pain would continue to improve with time. (Ex. JE7, p. 277). He stated Claimant had reached maximum medical improvement and recommended a functional capacity evaluation. (*Id.*). Dr. Bollier opined Claimant sustained 6% upper extremity impairment. (*Id.*).

Claimant presented to Dr. Darin Smith at Mercy Neurosurgery Clinic on June 16, 2020, for evaluation of his neck pain. (Ex. JE9, p. 601). Dr. Smith stated Claimant's neck issues were degenerative and not caused by the work injury but seemed to have been lit up. (Ex. JE9, p. 605). Dr. Smith did not recommend surgery. (*Id.*).

On August 26, 2020, Claimant presented to Dr. Chad Abernathey at Cedar Neurological Surgeons for a second neurosurgical opinion. (Ex. JE10, p. 607). Dr. Abernathey reviewed the MRI imaging and noted only mild degenerative changes; he thought the radiology interpretation overemphasized the actual results. (*Id.*). Dr. Abernathey stated there was no significant evidence on his MRI that there was a neurologic issue or cervical spine issue that required surgery. (*Id.*). He recommended Claimant follow up with his orthopedic physician. (Ex. JE10, p. 608).

On October 27, 2020, Claimant presented to Dr. James Nepola at the University of Iowa Hospitals & Clinics reporting right shoulder pain/stiffness. (Ex. JE7, p. 289). Dr. Nepola reviewed an updated MRI and noted a rotator cuff tear that was not terribly retracted or atrophic. (Ex. JE7, p. 291). He recommended a right shoulder arthroscopy to evaluate for arthritis and potentially repair the rotator cuff tear. (*Id.*). Claimant underwent a right revision rotator cuff repair with Dr. Nepola on November 4, 2020. (Ex. JE7, p. 293).

Claimant followed up with Stephanie Henrickson, ARNP, at Iowa City VA Psychiatry on January 19, 2021, for his depression. (Ex. JE1, p. 30). Claimant stated: "I'm doing pretty good." (Ex. JE1, p. 31). He was no longer taking trazodone. (*Id.*). Claimant denied any issues with depression, anxiety, anger, agitation, or irritability. (*Id.*). Claimant noted he was getting 6-8 hours of sleep at night and his "sleep isn't too bad." (*Id.*).

When Claimant followed up with Dr. Nepola on March 2, 2021, he reported neck pain. (Ex. JE7, p. 317). Claimant stated his neck pain began at the same time as his work injury on September 9, 2019; he reported the neck pain to Dr. White. (*Id.*). Dr. Nepola stated Claimant was progressing as expected related to his right shoulder, but Claimant had a neck problem. (Ex. JE7, p. 319). He referred Claimant to an orthopedic spine surgeon to address causation. (*Id.*).

On March 17, 2021, Claimant presented to Dr. Cassim Igram at the University of Iowa Hospitals & Clinics reporting central neck pain with radicular symptoms to the occipital scalp and right shoulder. (Ex. JE7, p. 321). He noted headaches and intermittent numbness in the bilateral upper extremities when his pain was severe. (*Id.*). Dr. Igram noted the pain was most likely related to the work injury. (Ex. JE7, p. 323). He reviewed the imaging and noted degenerative changes. (*Id.*). Dr. Igram stated Claimant did not need surgery and recommended physical therapy. (*Id.*).

On July 27, 2021, Claimant presented to Dr. Matthew Howard at the University of Iowa Hospitals & Clinics for a neurosurgeon evaluation. (Ex. JE7, p. 382). Claimant reported neck pain, bilateral shoulder pain, and left arm numbness and tingling. (*Id.*). Dr. Howard stated Claimant's MRI did not show any obvious compression of the thecal sac or exiting nerve roots; therefore, he did not believe Claimant's symptoms were related to the cervical spine. (Ex. JE7, p. 383). Dr. Howard recommended Claimant follow up with pain management and orthopedics. (*Id.*).

Claimant continued to report his right shoulder felt good to Dr. Nepola on August 10, 2021. (Ex. JE7, p. 385). Dr. Nepola stated Claimant had reached maximum medical improvement. (Ex. JE7, p. 387). He stated he spoke to Dr. Pugley who had agreed to provide another opinion related to Claimant's cervical

spine. (*Id.*). Dr. Nepola stated Claimant could "lift what he is able to" and recommended no repetitive reaching out away from the body or above shoulder height. (*Id.*). Dr. Nepola opined Claimant sustained 16% right upper extremity impairment. (*Id.*).

Claimant reported to Ms. Henrickson on August 23, 2021 that he was "not doing too bad." (Ex. JE1, p. 46). He reported anxiety 4 times a week and that he had more issues lately. (*Id.*). Claimant stated he was sleeping 9 hours a night. (*Id.*). Ms. Hendrickson increased Claimant's fluoxetine to 80 mg daily, and recommended psychotherapy but Claimant denied any need for psychotherapy. (Ex. JE1, p. 48).

On August 31, 2021, Claimant presented to Dr. Andrew Pugely for evaluation of his neck pain, bilateral shoulder pain, and right arm numbness and tingling. (Ex. JE7, p. 388). Dr. Pugely recommended C3-4 anterior cervical discectomy and fusion. (Ex. JE7, p. 392). Claimant underwent surgery with Dr. Pugely on October 18, 2021. (Ex. JE7, p. 399).

When Claimant returned to Dr. Pugely on April 21, 2022, he reported his bilateral arm symptoms had completely resolved. (Ex. JE7, p. 429). He had minimal neck pain and had returned to work without issues. (*Id.*). Dr. Pugely released Claimant without restrictions. (Ex. JE7, p. 431).

Claimant reported he was doing pretty good to Ms. Henrickson on May 9, 2021. (Ex. JE1, p. 65). He noted he had started a job at Meth-Wick. (*Id.*). Ms. Henrickson decreased the fluoxetine prescription to 40 mg daily. (Ex. JE1, p. 68).

On May 19, 2022, Claimant returned to the UIHC Spine Clinic reporting 2-3 weeks ago he started having recurrent neck pain and pain between his shoulder blades. (Ex. JE7, p. 434). He denied any specific inciting event. (*Id.*). A cervical spine MRI was recommended. (Ex. JE7, p. 436). On June 27, 2022, Claimant underwent a cervical spine MRI that indicated no acute changes. (Ex. JE7, p. 446).

Claimant reported to Ms. Henrickson on August 22, 2022, that he had stopped taking fluoxetine and he was "doing good." (Ex. JE1, p. 75). He noted occasional episodes of anxiety at work. (*Id.*). He denied depression or irritability. (Ex. JE 1, p. 77). Claimant stated his sleep was good and he was sleeping 6-8 hours per night. (Ex. JE1, p. 75). Ms. Henrickson recommended Claimant continue without medication. (Ex. JE1, p. 77).

Claimant returned to Dr. Pugely on August 25, 2022, reporting he was doing well for a couple months after surgery but now he was declining. (Ex. JE7, p. 454). Dr. Pugely reviewed updated MRI imaging and noted there was one small focus that could possibly explain his symptoms at the C5-6 disc based on the left. (Ex. JE7, p. 455). He recommended an injection at the C5-6 area as well as x-rays and a

CT. (*Id.*). Claimant underwent a steroid injection on September 16, 2022, and reported no improvement. (Ex. JE7, p. 466).

Claimant contacted Ms. Henrickson on October 11, 2022, stating there were "medical changes" with his neck and they were causing issues. (Ex. JE1, p. 79). Claimant wanted to restart the fluoxetine. (*Id.*). Ms. Henrickson agreed and stated Clamant could restart the fluoxetine that he had leftover at 40 mg daily. (*Id.*).

When Claimant returned to Dr. Pugely on December 13, 2022, Dr. Pugely noted Claimant had been evaluated by the shoulder team and hand team, had undergone diagnostic injections, and reported ongoing symptoms. (Ex. JE7, p. 501). Dr. Pugley offered a C5-6 and C6-7 anterior cervical discectomy and fusion and Claimant wanted to proceed. (Ex. JE7, p. 504).

Claimant underwent an anterior cervical diskectomy and fusion at C5-7 with Dr. Pugely on February 24, 2023. (Ex. JE7, p. 509). On April 11, 2023, Claimant reported to Dr. Pugely that he was doing well and had no neck pain or radicular pain. (Ex. JE7, p. 554).

III. Expert Opinions

Claimant underwent an independent medical examination with Dr. Joseph Chen on February 13, 2023. (Ex. C, p. 16). Dr. Chen opined Claimant's diagnosis was chronic myofascial pain arising from his neck and shoulder muscles. (Ex. C, p.

35). Dr. Chen opined Claimant's fusion in February 2023 was not causally related to the work injury in August 2019, and that his ongoing neck complaints were not related to the work injury. (*Id.*). Dr. Chen opined Claimant reached maximum medical improvement for his work-related right shoulder condition on August 10, 2021. (Ex. C, p. 37). Dr. Chen stated Claimant sustained 9% right upper extremity impairment. (Ex. C, p. 38). Dr. Chen agreed with the permanent restrictions assigned by Dr. Nepola. (*Id.*). He opined Claimant does not require further medical treatment causally related to the work injury. (*Id.*).

On June 23, 2023, Claimant underwent an independent medical examination with Dr. David Segal. (Ex. 1, p. 17). Dr. Segal opined Claimant sustained a cumulative injury to his right shoulder and neck due to his work as a drywall finisher for over 20 years. (*Id.*). He stated Claimant's job required repetitive forceful tasks in awkward positions with high-frequency forceful motion of the upper extremities. (*Id.*). Dr. Segal stated that Claimant then sustained a permanent aggravation of his preexisting depression. (*Id.*).

Dr. Segal opined Claimant reached maximum medical improvement for the right shoulder on August 10, 2021. (Ex. 1, p. 57). He stated Claimant sustained 21% right upper extremity impairment, which was 8% for range of motion deficits, 3% for strength deficits, 10% for a distal clavicle excision, and 2% for the biceps. (Ex. 1, pp. 57-58). Dr. Segal recommended permanent work restrictions of lifting

up to 20 pounds occasionally, pushing/pulling up to 30 pounds occasionally, rare overhead reaching, and no overhead lifting. (Ex. 1, p. 61).

In regard to the cervical spine, Dr. Segal opined Claimant reached maximum medical improvement on June 24, 2023. (Ex. 1, p. 64). He stated Claimant sustained 35% whole body impairment. (Ex. 1, p. 67). Dr. Segal recommended permanent work restrictions of lifting up to 30 pounds from floor to waist and 15 pounds from waist to chest and standing/walking for no more than 60 minutes at a time. (Ex. 1, p. 68).

Dr. Segal opined Claimant sustained reactive depression and anxiety, which was a permanent aggravation. (Ex, 1, p. 69). He stated that chronic pain was a significant psycho-social stressor. (Ex. 1, p. 72). Dr. Segal stated Claimant's mental health diagnosis had a substantial impact on his employability. (Ex. 1, p. 74). He believed Claimant had mild impairment in his ADLs, social functioning, concentration, and adaptation; therefore, he sustained 18% whole body impairment due to his mental health condition. (Ex., 1, p. 76). Dr. Segal recommended permanent work restrictions related to his condition. (Ex. 1, p. 77).

Claimant underwent an independent mental health evaluation with Dr. Adam James Woods on August 8, 2023. (Ex. 2, p. 133). In his report dated August 16, 2023, Dr. Woods assessed Claimant with adjustment disorder with mixed anxiety and depressed mood. (Ex. 2, p. 136). He noted that adjustment disorder is correctly

diagnosed when a person had stable mental health symptoms and then some external event occurred, and the person begins experiencing mental health symptoms above and beyond what they were experiencing before. (Ex. 2, pp. 136-37). Dr. Woods noted Claimant didn't require mental health treatment until 2017. (Ex. 2, p. 137). He stated that Claimant was having minimal mental health symptoms until the summer of 2019, when he started demonstrating worse psychiatric issues affecting multiple aspects of his life. (Ex. 2, p. 138). Dr. Woods stated the work injury "undoubtably was the 'identifiable stressor' which triggered this diagnosis." (*Id.*). He opined Claimant had Class 3 or Moderate Impairment under the AMA Guides, which he believed corresponded to a 40% impairment rating. (Ex. 2, p. 140).

Dr. Chen evaluated Claimant on August 19, 2023, for a supplemental independent medical examination. (Ex. C, p. 45). Dr. Chen stated his medical diagnosis was still chronic myofascial pain arising from the neck and shoulder muscles. (Ex. C, p. 56). Dr. Chen noted Claimant reported he was doing well with his neck but now he was having right shoulder pain. (*Id.*). Dr. Chen stated Claimant had a pattern of doing well for several weeks after surgical intervention and then having episodes of acute worsening; he stated the symptoms were consistent with chronic myofascial pain that could wax and wane due to idiopathic or unknown causes. (Ex. C, p. 57). Dr. Chen stated he continued to believe that the

February 2023 spinal surgery was not causally related to the work injury. (*Id.*). He stated Claimant's MRI findings from November 2022 were unrelated to his work or work injuries as these were expected from the natural course of degenerative findings in a cervical spine of a working-aged adult. (Ex. C, p. 58). Dr. Chen stated Claimant did not require any additional medical treatment related to the work injury. (*Id.*).

Dr. Martin Carpenter provided an independent psychiatric record review opinion dated August 31, 2023. (Ex. D, p. 65). Dr. Carpenter opined Claimant did not sustain a psychological or mental health injury related to the alleged work injury. (Ex. D, p. 66). He stated the records demonstrate Claimant had preexisting depression and longstanding alcoholism. (*Id.*). Dr. Carpenter noted Claimant was diagnosed with adjustment disorder and depressed mood in 2017. (*Id.*). Dr. Carpenter opined Claimant did not sustain any permanent impairment since he did not sustain a mental injury related to the work injury and he did not require any permanent work restrictions. (Ex. D, p. 67).

ARGUMENT

I. The Commissioner did not commit an error of law in concluding that Claimant is not entitled to industrial disability benefits.

A. Preservation of Error

Defendant believes that Claimant preserved error regarding this issue.

B. Standard of Review

Defendant agrees with the Claimant's standard of review.

C. Argument

The Commissioner was correct in holding that Claimant is not entitled to industrial disability benefits. (App. Dec. p. 4). It was not an error of law or irrational, illogical, or wholly unjustifiable. Even through Claimant sustained an injury to his body as a whole, he is not entitled to industrial disability benefits based on Iowa Code § 85.34(2)(v), which states:

> If an employee who is eligible for compensation under this paragraph returns to work or is offered work for which the employee receives or would receive the same or greater salary, wages, or earnings than the employee received at the time of the injury, the employee shall be compensated based only upon the employee's functional impairment resulting from the injury, and not in relation to the employee's earning capacity.

IOWA CODE § 85.34(2)(v) (2019).

In Nusinovic the deputy held that the claimant was not entitled to industrial disability benefits. *Nusinovic v. UnityPoint Health-Waterloo*, File Nos. 19001748.01; 20009674.01; 20700556.01; 21700201.01; 21700202.01; 21700203.01 (Arb. Dec. Feb. 24, 2022). The claimant was working as a housekeeper at the time of the alleged injuries. *Id.* at *6. Following her release from medical treatment, the defendant offered the claimant a full-time

administrative assistant position where she would earn higher wages than at the time of the injury. *Id.* at *31. The position was within the permanent work restrictions imposed by Dr. Sassman. *Id.* The claimant declined the job "due to pain", although she never attempted the job. *Id.* Deputy Palmer held that the defendant had offered the claimant work at a greater wage than what she was earning at the time of the work injury that was consistent with her work restrictions, and she refused the work. *Id.* Therefore, she was only entitled to functional disability benefits. *Id.* The commissioner affirmed the decision on appeal. *Nusinovic v. UnityPoint Health-Waterloo*, File Nos. 19001748.01; 20009674.01; 20700556.01; 21700201.01; 21700202.01; 21700203.01 (App. Dec. July 19, 2022).

In contrast, the deputy in Newburry found the claimant was entitled to industrial disability benefits despite that defendant offered the claimant a job after the work injury. *Newburry v. Lutheran Home for the Aged Assoc.*, File No. 21000314.02 at *11 (Arb. Dec. Jan. 9, 2023). In that case, the deputy found that the job that was offered was not within the claimant's permanent work restrictions and the claimant was not physically capable of performing the position. *Id.* Since the claimant's work restrictions restricted him from performing the job, it was not an offer that was contemplated by Iowa Code § 85.34(2)(v). *Id.*

In this case, Defendants made a good faith offer to Claimant after Dr. Nepola imposed permanent work restrictions for his right shoulder. Defendants sent formal correspondence to Claimant on August 25, 2021, regarding the offer of the superintendent position. (Ex. 7, p. 294). Claimant was offered a position where he would earn $60,000 annually with benefits. (*Id.*). Claimant was also verbally offered the position of commercial estimator. (Hr. Tr. pp. 42-43; Ex. 7, p. 296). Claimant testified that he declined the job offers and stated he felt that he was not qualified to perform the positions. (Hr. Tr. pp. 42-43).

At the time of the work injury, Claimant's average weekly wage was $864.90, which equates to approximately $45,000 annually. The job offers were for greater wages than what Claimant was earning at the time of the work injury. Based on the fact that Defendants offered Claimant positions within his permanent work restrictions that paid the same or greater wages than his pre-injury position and he chose not to return to work with Defendants, he is not entitled to industrial disability benefits under § 85.34(2)(v).

Claimant chose to stop working for Defendants based on his own beliefs about his ability to perform the jobs. The reason for Claimant declining the jobs was not because they were outside of his permanent work restrictions. (*See* Hr. Tr. pp. 42-45). Claimant stated he did not feel capable of performing the jobs due to the "physical and mental demand of that job." (Hr. Tr. p. 43). Claimant never

attempted to perform the jobs. Clearly the employer believed Claimant had the necessary qualifications to successfully complete the jobs based on their good faith offers. The nature of the job was within the claimant's expertise—construction work. It was not a job that was completely and totally different from the nature of the claimant's past work experience.

Claimant asserts that he was not at maximum medical improvement for his neck and later underwent surgery on his neck; therefore, he was not physically able to perform the jobs that were offered to him. Defendants offered the jobs to Claimants in good faith. Had Claimant accepted a job and then needed to undergo surgery in the future, the employer's typical policies regarding medical leave would have been followed. Claimant continued to work for Defendants throughout the course of his medical treatment related to his right shoulder. There is no reason to believe that Claimant could not have continued working in a different position for Defendants during the course of his treatment for his neck.

Defendants made a good faith offer of employment within the field of Claimant's prior employment with Defendants. Claimant declined those offers. Pursuant to the plain language of Iowa Code § 85.34(2)(v), this disqualifies the claimant from an industrial disability award. The Commissioner's decision was not an error at law or irrational, illogical, or wholly unjustifiable under these facts and should be upheld.

II. There is substantial evidence to support the Agency's decision that Claimant's mental health injury should not be considered in an industrial disability analysis.

A. Preservation of Error

Defendant believes that Claimant preserved error regarding this issue.

B. Standard of Review

Defendant agrees with the Claimant's standard of review.

C. Argument

Defendants maintain that Claimant is not entitled to an industrial disability award based on Iowa Code § 85.34(2)(v) and the offers of employment. If it were determined that Claimant is entitled to industrial disability, the Agency adequately took Claimant's alleged mental health condition into account when it assessed the extent of industrial disability. The Agency noted that Dr. Woods, Claimant's mental health expert, did not assign any permanent work restrictions related to his alleged mental injury. (App. Dec. p. 3). Dr. Woods also notes that the AMA Guides do not provide numerical ratings. (Ex. 2, p. 140).

Claimant asserts that Dr. Woods statements that "it is highly doubtful that Mr. Wooff will be able to work a physical labor job ever again" equates to permanent work restrictions. (Clmt Br. p. 34). Defendants dispute that this equates to permanent work restrictions. In the context of Dr. Woods' report, it appears that

Dr. Woods is making an assessment of the claimant's ability to return to gainful employment in light of the entirety of his alleged injuries, which is an assessment to be made by the Commissioner.

Claimant further asserts that the Agency did not adequately take Dr. Woods' 40% whole-body impairment rating related to his mental health into account in the industrial disability analysis. However, the Commissioner specifically addressed this in his response to Claimant's Application for Rehearing and stated that Dr. Woods impairment rating was considered and rejected. (Ruling on Motion for Rehearing p. 4).

The Commissioner considered Dr. Woods opinion in the evaluation of the industrial disability award, despite that the Commissioner ultimately determined that Claimant is not entitled to an industrial disability evaluation. The medical records support the conclusion that Claimant had returned to his preinjury mental health baseline, and he did not require any permanent work restrictions related to his alleged mental health injury.

III. The Agency did not err as a matter or law or use Agency expertise in determining the extent of permanent impairment.

A. Preservation of Error

Defendant believes that Claimant preserved error regarding this issue.

B. Standard of Review

Defendant agrees with the Claimant's standard of review.

C. Argument

The Agency correctly determined the extent of Claimant's functional disability related to his right shoulder, neck, and mental injuries (although the Agency incorrectly added the functional disability amounts together, which the District Court addressed in its Order regarding the calculation of whole body impairment filed on August 5, 2025). The Agency did not improperly use its expertise in determining impairment. The Agency must use logic and reasoning, rely upon its prior interpretation of the AMA Guides, and analyze the facts of the case at hand to determine which impairment rating to adopt. The Agency did nothing more in this case.

The Agency adopted Dr. Chen and Dr. Nepola's opinions that Claimant sustained 6% upper extremity impairment for his right shoulder based on his loss of range of motion. (App. Dec. p. 5). The Agency also adopted Dr. Chen's opinion that Claimant sustained an additional 3% upper extremity impairment related to the distal clavicle resection. (*Id.*). This is consistent with the Agency's prior case law regarding distal clavicle resections.

Claimant asserts that Dr. Segal's impairment rating should be adopted. Dr. Segal stated Claimant sustained 21% right upper extremity impairment, which was 8% for range of motion deficits, 3% for strength deficits, 10% for a distal clavicle excision, and 2% for the biceps. (Ex. 1, pp. 57-58). Dr. Segal is the only physician to opine that Claimant sustained specific impairment to the biceps. Claimant did not undergo any surgery specifically to the biceps. There is no indication that the impairment rating to the shoulder does not encapsulate all impairment related to the right shoulder injury. Dr. Segal's impairment rating to the biceps is an outlier in this case.

The additional 10% upper extremity impairment for the distal clavicle excision is inconsistent with prior Agency case law. The Commissioner held in Jay that it is not consistent with the AMA Guides to add 10% impairment for a distal clavicle excision. The Commissioner stated:

> Table 16-27 of the AMA Guides governs impairment of the upper extremities after arthroplasty of specific bones and joints. Under Table 16-27, a distal clavicle excision is assigned ten percent impairment. Page 498 of the AMA Guides directs the examining physician as follows:
>
> Conditions not previously described that can contribute to impairments of the hand and upper extremity include bone and joint disorders (Section 16.7a), presence of resection or implant arthroplasty (Section 16.7b), musculotendinous disorders (Section 16.7c) and tendinitis (Section 16.7d), and loss of strength (Section 16.8). The severity of the impairment due to these disorders is rated separately according to Tables 16-19 through 16-30 and then

> multiplied by the relative maximum value of the unit involved as specified in Table 16-18 ….
>
> Under Table 16-18, the appropriate multiplier for the acromioclavicular joint is 25 percent.

Jay v. Archer Skid Loader Service, LLC, File No. 19003586.01, 2022 WL 17078713 at *7 (Iowa Workers' Comp. Comm'n App. Dec. Aug. 23, 2022). The Commissioner held that after applying the 25% multiplier to the 10% impairment, the result was 2.5% impairment related to the distal clavicle excision. (*Id.*).

The Commissioner's interpretation of this portion of the Guides was affirmed by the district court on appeal. *Jay v. Archer Skid Loader Service, LLC*, Case No. CVCV064318, at p. 7 (Iowa Polk Cty District Ct. May 22, 2023). The district court held that the Commissioner can determine its own impairment rating if none of the physician ratings in evidence are consistent with the AMA Guides. (*Id.* at p. 6). However, the district court held that the impairment rating must be rounded to a whole number based on the instructions in the AMA Guides. (*Id.* at p. 7). Therefore, the 2.5% rating for the distal clavicle resection would be rounded up to 3% impairment. (*See id.*). The Agency has adopted this approach in subsequent cases as well. *See Vance v. Delaney Concrete Construction Co., Inc.*, File No. 19004997.01 at *12 (Iowa Workers' Comp. Comm'n Arb. Dec. May 2, 2023); *Brown v. Estes Express*, File No. 20014133.01 at *5 (Iowa Workers' Comp. Comm'n Arb. Dec. Sept. 6, 2023); *Gebert v. Bridgestone America, Inc.*, File No.

23700429.01, 651494.01, 220005546.01 at *8 (Iowa Workers' Comp. Comm'n Arb. Dec. April 25, 2024).

The Commissioner did not use its own expertise in coming to the conclusion regarding the extent of functional disability. The Commissioner didn't ignore all of the physician opinions in evidence and determine a totally different impairment rating. The Commissioner adopted a portion of Dr. Nepola's impairment rating but determined his opinion regarding the extent of disability related to the distal clavicle excision was not consistent with the AMA Guides. "The commissioner may accept or reject the expert opinion in whole or in part." *Schutjer v. Algona Manor Care Ctr.*, 780 N.W.2d 549, 560 (Iowa 2010) (emphasis added). That is exactly what the Commissioner did in this case—he rejected and accepted portions of Dr. Nepola's impairment rating. He accepted the entirety of Dr. Chen's impairment rating. The AMA Guides are specifically incorporated by statute and administrative rule, and it would not be appropriate to ignore their provisions.

The claimant's argument that the Agency cannot take into consideration the correct use of the AMA Guides or its prior case law regarding the use of the AMA Guides would lead to irrational and illogical results. The Agency would be required to adopt any rating that a party put into evidence regardless of whether it is logical or consistent with the AMA Guides. The claimant's arguments in the Barry case are similar as in this case. The claimant asserted that the Commissioner

violated Iowa Code § 85.34(2)(x) when it rejected his expert's opinion because the impairment ratings were not consistent with the AMA Guides. *Barry v. John Deere Dubuque Works*, 995 N.W.2d 516 (Table), 2023 WL 3612394 at *3 (Iowa Ct. App. 2023). The claimant asserted that the statute prohibited the Commissioner from finding that the expert misapplied the AMA Guides when weighing credibility and the Commissioner abused his discretion by used the AMA Guides. *Id.* The Iowa Court of Appeals rejected this argument and held that the Commissioner did not abuse his discretion by relying on the AMA Guides to determine expert credibility. *Id.*

Similarly in this case, the Commissioner did not err in relying on the AMA Guides to determine the extent of functional disability. If the Commissioner is not allowed to utilize the Guides, it will lead to unpredictable and absurd results. The intent of the revisions to Iowa Code § 85.34(2)(x) was the prevent unpredictable results that were not consistent with the Guides. The Commissioner's reliance on the Guides is consistent with the statute.

The Commissioner did not err in rejecting Dr. Woods 40% body as a whole impairment rating. Dr. Woods stated that the AMA Guides do not provide impairment ratings for mental conditions. Yet, he provided his own estimation of permanent impairment of 40%. (Ex. 2, p. 140). Given the other evidence in the record, a 40% whole body impairment rating is clearly inflated.

The medical records document that Claimant's mental health condition has improved since 2019, and he has returned to his preinjury baseline. Claimant is currently taking fluoxetine, which is the same medication that he took prior to the work injury. Claimant's alleged mental condition has not caused any significant functional disability or impacted his activities of daily living based on the records in evidence. Claimant declined psychotherapy that was suggested by Dr. Hendrickson on numerous occasions. He weaned off fluoxetine by August 22, 2022 and stated he was "doing good." (Ex. JE1, p. 75). While Claimant began taking some of his leftover fluoxetine in October 2022, he did not experience any significant worsening of his medical condition. (*See* Ex. JE1, p. 79). He remained at baseline.

The medical records do not demonstrate any significant disability due to Claimant's mental condition. Dr. Woods impairment rating of 40% is not consistent with the evidence in the record or the AMA Guides. It is within the Agency's discretion to accept or reject an expert opinion, in whole or in part. *See Schutjer v. Algona Manor Care Ctr.*, 780 N.W.2d 549, 560 (Iowa 2010). The Commissioner analyzed the record and expert opinions and rejected Dr. Woods impairment rating. The Agency was correct in determining that Claimant is not entitled to any functional disability due to his alleged mental health condition and the Commissioner's decision should be affirmed.

CONCLUSION

The Commissioner's Appeal Decision and the District Court's Ruling should be upheld. The Commissioner did not err in concluding that Claimant is not entitled to industrial disability benefits as he was offered suitable work with the same employer. The Agency did not improperly utilize Agency expertise in determining the award of functional disability benefits related to the right shoulder or neck.

ORAL ARGUMENT

Appellant believes oral argument is not necessary for resolution of the issues raised in this appeal.

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CERTIFICATE OF FILING/SERVICE

I hereby certify that a true and accurate copy of this instrument has been and will be filed electronically with the Clerk of the Iowa Supreme Court and forwarded to all counsel via the electronic filing system on this 28th of November, 2025, and by U.S. Mail for any party not registered to receive notice of filings via the ECF process.

/s/ Kent M. Smith

CERTIFICATE OF COMPLIANCE

1. This brief complies with the type-volume limitation of Iowa Rule of Appellate Procedure 6.903(1)(i)(1) because it contains 7,300 words, excluding the parts of the brief exempted.

2. This brief complies with the typeface requirement of Iowa Rule of Appellate Procedure 6.903(1)(g)(1) because it has been prepared in proportionately spaced typeface using Microsoft Word 2023 in Times New Roman 14-point type.

/s/ Kent M. Smith